

Criteria	Yes	No
Diagnosis Discrepancy	☒	☒
Primary Tumor Site Discrepancy	☒	☒
HPAA Discrepancy	☒	☒
Prior Malignancy History	☒	☒
Dual/Synchronous Primary	☒	☒
Case is (circle):	QUALIFIED	DISQUALIFIED
Reviewer Initials: <i>RB</i>	Date Reviewed: <i>10/5/11</i>	

UID: 8435621F-F2C1-492A-A3E8-E6FAE9BD4351
TCGA-EL-A3CR-01A-PR

Redacted

Pathology Report

DOB:
Physician:

Sex: F

Collected:

Received:

Case type: Surgical Case

Accession:

DIAGNOSIS

(A) THYROID, TOTAL THYROIDECTOMY:

PAPILLARY CARCINOMA OF THE RIGHT THYROID, FOLLICULAR VARIANT, INFILTRATING THE ANTERIOR SOFT TISSUE AND INVOLVING THE ANTERIOR AND POSTERIOR MARGINS (3.5 CM IN MAXIMUM DIMENSION).

FOUR OUT OF SIX LYMPH NODES WITH METASTATIC PAPILLARY CARCINOMA (4/6).

(B) LYMPH NODES, RIGHT NECK, LEVEL 2:

Eight lymph nodes, no tumor present (0/8).

(C) LYMPH NODES, RIGHT NECK, LEVEL 3:

Six lymph nodes, no tumor present (0/6).

(D) LYMPH NODES, RIGHT NECK, LEVEL IV:

FOUR OUT OF EIGHT LYMPH NODES WITH METASTATIC PAPILLARY CARCINOMA (4/8).

(E) LYMPH NODES, RIGHT NECK, LEVEL V:

Four lymph nodes, no tumor present (0/4).

Entire report and diagnosis completed by

100-0-3

Path: carcinoma, papillary, follicular variant

834013

CQCF: carcinoma, papillary thyroid

826013

Site: • thyroid, nos C73.9

hw
10/1/11

GROSS DESCRIPTION

(A) TOTAL THYROIDECTOMY AND RIGHT PARATRACHEAL NODE DISSECTION - A product of total thyroidectomy and right fibroadipose tissue and soft tissue with overall measurement of 10.0 x 3.5 x 2.5 cm. The right lobe appears to be regular, firm to mostly adherent to soft tissue and measures 3.5 x 3.5 x 2.5 cm. The isthmus appears nodular, but grossly unremarkable. The left lobe is 3.5 x 2.0 x 2.0 cm and appears unremarkable. Tumor appears to abut the margin of resection anteriorly and posteriorly. On sectioning, the 3.0 x 2.5 x 2.5 cm light tan hard mass occupying the entire right lobe with extension to the anterior soft tissue is noted. The tumor appears to be well circumscribed, but extending into adjacent soft tissue. No abnormalities and isthmus or the right lobe were seen. The specimen is submitted entirely in A1-A12, sequential sectioning of the right lobe from anterior to posterior; A13, isthmus; A14, A15, representative section of the left lobe. Small nodules in the cervical lymph nodes were identified; A17, other soft tissue with suspected lymph nodes are submitted under A18.

(B) RIGHT NECK LEVEL II - An irregular fragment of fibrous and adipose tissue (7.0 x 4.0 x 1.0 cm). Identified are several lymph nodes ranging in size from (0.2 x 0.2 x 0.2 to 1.0 x 0.8 x 0.8 cm).

SECTION CODE: B1, four lymph nodes; B2, one lymph node; B3, two lymph nodes.

(C) RIGHT NECK LEVEL III - An irregular fragment of fibrous and adipose tissue (6.0 x 3.0 x 1.0 cm). Identified are five lymph nodes ranging in size (0.3 x 0.2 x 0.2 to 1.0 x 0.8 x 0.3 cm).

SECTION CODE: C1, one lymph node; C2, two lymph nodes; C3, two lymph nodes.

(D) RIGHT NECK DISSECTION LEVEL IV - An irregular fragment of fibrous and adipose tissue (6.0 x 3.5 x 0.8 cm). Identified are six lymph nodes ranging in size (0.2 x 0.2 x 0.2 to 2.0 x 1.5 x 1.0 cm).

Surgical Pathology Report

Department of Pathology,

DOB:
Physician:

Sex: F

Collected:

Pathologist:

Accession:

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SECTION CODE: D1, one lymph node bisected; D2, one lymph node bisected; D3, one lymph node bisected – grossly positive; D4, one lymph node bisected; D5, one lymph node; D6, one lymph node.

(E) RIGHT NECK LEVEL V – An irregular fragment of fibrous and adipose tissue (5.0 x 5.0 x 1.5 cm). Identified are four lymph nodes ranging in size (0.3 x 0.3 x 0.2 to 0.6 x 0.4 x 0.4 cm).

SECTION CODE: E1, two lymph nodes; E2, two lymph nodes.

CLINICAL HISTORY

History of thyroid cancer.

SNOMED CODES

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File under: Pathology	